



February 22, 2023

Outpatient treatment for mild - moderate COVID WITHOUT hypoxemia or increase oxygen requirement from baseline:

Paxlovid (Nirmatrelvir-Ritonavir)

- Given within 5 days of symptom onset (earlier the better)
- It has been shown to reduce hospitalization and death in unvaccinated individuals with risk factors for severe disease.
- In vaccinated individuals with COVID-19, it decreases ED visits, hospitalization, and death.
- Contraindications to Paxlovid:** GFR <30 or severe hepatic impairment (Child- Pugh Class C- advanced cirrhosis)

Special considerations:

- Plavix:**
Patients who are on Plavix for stent (within 6 weeks of stent placement), recent stroke (within 3 months)- should not be prescribed Paxlovid because of risk of thrombosis, since Paxlovid decreases efficacy of Plavix. Consider alternative treatment- **Remdesivir**.
- Direct oral anticoagulants (DOACs)**—dabigatran (Pradaxa), rivaroxaban (Xarelto), apixaban (Eliquis), edoxaban (Savaysa), and betrixaban (Bevyxxa)
Patients on DOACs for high-risk blood clots (clot within past 6 months, clot at any time in past when anticoagulation was interrupted, active cancer with clot at any point in cancer journey or diagnosis of antiphospholipid antibody syndrome)- should **not** be given Paxlovid because adjusting dose of DOAC (decreasing dose) may cause thrombosis, and if no dose adjustment, patient may have increased risk of bleeding.
- Warfarin**
Patients on warfarin can continue as prescribed, no dose adjustment needed. INR will need to be monitored sooner. Patient can notify their anticoagulation clinic to get advice on next INR date.
- CYP3A4 inducer** (phenobarbital, phenytoin, rifampicin, St. John's Wort, etc.)
Patients on these medications/supplements should NOT be prescribed Paxlovid because it will not be effective and will increase risk of drug resistance.

Remdesivir

- Given within 7 days of symptom onset - requires daily IV infusion for 3 days.
No contraindications or lab work necessary prior to treatment.

Pregnant patients:

Paxlovid and **Remdesivir** are currently being used in pregnant patients, but studies are ongoing. So far, data has not shown any adverse pregnancy outcomes.

Molnupiravir (not preferred treatment) – only prescribe if no alternative available and recommended by ID

- Not preferred treatment due to reproductive concerns and teratogenicity
- Contraindicated in age <18 y/o, pregnancy, and lactation
- Given within five days of symptom onset (earlier the better)
- No dose adjustment is necessary based upon kidney or hepatic impairment
- No drug-drug interactions

COVID Prophylaxis:

Tixagevimab-cilgavimab (Evusheld) is no longer authorized for prevention of COVID-19. It is not active against current circulating Omicron subvariants.

**Special thanks to Julie Amio and the VMC pharmacy team*

GCN Tips:

No Kaiser backdrop, no problem. You can blur your background during video visits on GCN by clicking on this icon in the video visit windows.

Click on,  or 

Save time during your GCN visits

Instead of counseling patients on the phone, insert smart text instructions via "Pt instructions" in Health Connect. You can create speed buttons using the wrench for commonly used instructions or type in SmartText box to search for instructions.

